

Response and Proposed Allocation of Questions

Request for Medical Information dated 31 July 2026

Employee: Cory Lea Shepherd, AO3, Switchboard Services, Logan Hospital

Reference: MSH-INJ-5795 · CLM-317073

Request signed by: Mr Scott Hughes, Director, Corporate Services, LBHS — 31 July 2026

Response date: _____

PART 1 — THE ORIGIN OF THE REQUEST, AND THE TEST APPLIED

1.1 THE ORIGIN AND STATED PURPOSE OF THE EMPLOYEE CAPABILITIES CHECKLIST

The letter of 31 July 2026 records the origin of the checklist in the Health Service's own words:

"your recently completed Employee Capabilities Checklist (ECC) (Attachment 1) completed by Dr Day Hong Ma on the 3 July 2026 which was provided to us to facilitate a Return to Work Plan."

That is correct, and the sequence was as follows.

- (a) On 2 July 2026 I was told by the Manager, Switchboard Services that my return to work could not be facilitated until a completed Employee Capabilities Checklist was provided, and that it was required before 2:00pm on Friday 3 July 2026.
- (b) I obtained the checklist and provided it on 3 July 2026. It certified me fit to work with restrictions.
- (c) Within the hour I was told I could not attend the workplace, and my rostered shifts were charged to my accrued leave. I have not worked since.

The checklist was therefore required by the Health Service, for a purpose the Health Service has itself stated — to facilitate a return to work — and, once provided, became the basis on which work was refused. It did not certify me unfit. It certified me fit with restrictions.

I record this because it bears directly on the present request. The Health Service now seeks further information about a document it required me to obtain in order to work, and which certified that I could.

I am willing to provide clarification of that document. I ask that the clarification be directed to identifying the adjustments which allow me to work — the purpose for which the document was obtained, the purpose stated in the letter of 31 July 2026, and the purpose contemplated by Human Resources Policy G03.

1.2 THE TEST APPLIED

The letter to Dr Ma states the purpose of the request:

"to identify and implement any appropriate workplace controls or reasonable adjustments to support a safe and sustainable return to his substantive position."

Human Resources Policy G03 (QH-POL-210), which the Health Service has cited, sets the limit at clause 3:

"Reasonable adjustment may need to be supported by medical or rehabilitation information, where necessary, to either identify required workplace adjustments and/or assess the suitability of proposed adjustments."

I have applied that test to each question: is this information necessary to identify a required adjustment, or to assess the suitability of a proposed one — and is the person being asked the right person to answer it?

Applying that test, the nine questions divide four ways. I am not declining to provide information. I am proposing that each question be directed to whoever is able to answer it.

1.3 INFORMATION THE HEALTH SERVICE ALREADY HOLDS

Much of what is sought is already in the Health Service's possession.

The Employee Capabilities Checklist of 3 July 2026 states — as quoted in the Health Service's own letter of 31 July 2026 — that I have a diagnosis of Major Depressive Disorder with anxious distress, "with symptoms exacerbated on exposure to identified workplace stressors, including complaint handling, being held accountable and blamed for the failures of others, and unpredictable rostering"; that the injury is a "work-related psychological injury"; that "unresolved workplace matters involving line management" is a further stressor; that my "working memory is affected under stress"; and that I should not undertake complaint-handling duties.

In addition:

- (a) Attachment 2 to the letter to Dr Ma is the Health Service's own Role Description for the position. It contains no complaint-handling responsibility.
- (b) The letter of 31 July 2026 states the Health Service's own position that complaints received by Switchboard employees are escalated to the Client Liaison Officer and/or the Manager, Switchboard Services.
- (c) The letter of 7 July 2026 acknowledged that my restrictions were already being accommodated.
- (d) I worked in my substantive role, under the existing reporting arrangements and on adjusted hours, for approximately twelve months to 3 July 2026. The Health Service's own rosters and payroll records evidence that period.

That material already answers Question 5, and goes a considerable way toward answering Questions 1(d), 3, 4, 6 and 8. What genuinely remains outstanding is narrow, and I identify it at Part 3.

I raise this because I have now been excluded from the workplace for five weeks while information the Health Service largely already holds is sought again.

PART 2 — PROPOSED ALLOCATION

Q	SUBJECT	PROPOSED RESPONDENT
1(a)	Date of first diagnosis	Reframed — Dr Ma, with clinical context
1(b)	Clinical basis for the causal link	Withdrawal requested
1(c)	Self-report v clinical assessment; other reports held	Withdrawal requested
1(d)	Foreseeable risk; controls medically necessary	Dr Ma (GP)
2	Following directions; performance and conduct discussions	Health Service — threshold question
3	Fitness under the existing reporting arrangements	Health Service — risk assessment, then treating psychiatrist
4	Restrictions, adjustments, duration and review date	Dr Ma (GP)
5	Meaning of “complaint handling”	Health Service — already answered, then Dr Ma
6	Requirements of the role without modification	Health Service — identify genuine occupational requirements, then GP
7	“Working memory affected under stress”	Treating psychiatrist
8	Tasks and environments that exacerbate the condition	Dr Ma (GP)
9	If unable to accommodate, whether safe return is possible	Health Service — onus is the employer's

Of the nine questions, four are not medical questions. They are questions the Health Service must answer itself before any clinician can usefully be asked anything.

PART 3 — QUESTIONS FOR MY PRACTITIONERS

3.1 FOR DR MA — I will obtain answers to these

Q1(d) Whether exposure to the identified stressors presents a foreseeable risk to my health or safety on return, and what specific workplace controls or adjustments are medically necessary to manage that risk.

Q4 The restrictions or adjustments recommended, the clinical basis in functional terms, and the anticipated duration and review date.

Q5 What specific activities are encompassed within “complaint handling” — as a matter of clinical restriction. The operational meaning has already been stated by the Health Service and is dealt with at Part 4.3.

Q8 Specific tasks, situations or environments which may exacerbate my condition.

Each is necessary to identify a required adjustment or to assess the suitability of a proposed one. Each is within a general practitioner's scope.

3.2 FOR A TREATING PSYCHIATRIST — I am willing to obtain this

Q7 What "working memory is affected under stress" means in functional terms, including how it may affect capacity under normal workplace demands and the circumstances likely to cause difficulty.

This is a legitimate adjustment question, but a functional cognitive opinion is properly a psychiatrist's rather than a general practitioner's. Dr Ma can address it in general terms only.

So that it can be arranged, please confirm:

- (a) that the Health Service presses Question 7, and identify any part of Questions 1(d), 4 or 8 on which it requires a specialist rather than a general practitioner's opinion;
- (b) that the Health Service will meet the cost, consistent with its confirmation that it will meet my reasonable costs; and
- (c) that it accepts the resulting timeframe. A psychiatric review cannot be arranged within seven days and in the ordinary course will take considerably longer.

Consistent with option 1, I will obtain that opinion and provide it. Its scope will address my current capacity, functional restrictions and recommended adjustments. It will not address the aetiology of my condition, which is a matter in issue in WC/2024/227 and is not necessary for the stated purpose of this inquiry.

The sequence this requires is worth stating, because it bears on the timeframe. A referral to a psychiatrist is made by my general practitioner. So the order is: the consultation with Dr Ma is rebooked and held; Dr Ma answers the questions within his scope and, if the Health Service presses Question 7, provides a referral; an appointment with the psychiatrist is then obtained; and the opinion follows. None of that can occur within seven calendar days, and the first step cannot sensibly occur until the Health Service has confirmed which questions it presses.

I note that a request to my own treating practitioners rests on a different footing from an independent medical examination, which is governed by chapter 3 part 8 division 5 of the Public Sector Act 2022 and Directive 04/24, and which has not been invoked.

PART 4 — QUESTIONS FOR THE HEALTH SERVICE

These four questions ask a doctor to answer matters which the Health Service's own policy and the Work Health and Safety Act 2011 place with the Health Service. I ask that the Health Service answer them, after which the properly medical component of each can be put to my practitioners.

4.1 QUESTION 2 — is there a performance or conduct process?

The question asks whether I can "follow a reasonable and lawful direction issued by [my] supervisor" and "participate in discussions in relation to [my] workplace performance and/or conduct."

These are not questions about capacity to perform the genuine occupational requirements of my role. They concern readiness for a management process. Either such a process is on foot concerning me — in which case I am entitled to be told of it, given particulars, and afforded procedural fairness — or there is not, in which case the questions do not serve the stated purpose of this inquiry.

→ **FOR THE HEALTH SERVICE: please confirm whether any performance or conduct process concerning me is on foot or contemplated, and if so provide particulars.**

→ **THEN, IF APPROPRIATE, FOR MY PRACTITIONER: "Does the employee have capacity to participate in ordinary workplace meetings and supervision, and are any supports required?"**

The premise of Question 2 is separately incorrect and is addressed at Part 5.

4.2 QUESTION 3 — is the workplace safe?

The question asks whether I am "medically fit to return to their substantive role under the existing reporting arrangements, including working with and reporting to their current line manager."

This asks a treating doctor to certify that a workplace arrangement is safe. Under section 19 of the Work Health and Safety Act 2011 that duty rests with the Health Service. Under the Managing the risk of psychosocial hazards at work Code of Practice 2022, a psychosocial hazard is a feature of the work, to be identified and controlled by the person conducting the business or undertaking. G03 clause 1 requires that "proper consideration must be given to the Managing the risk of psychosocial hazards at work Code of Practice 2022, to ensure the process of providing reasonable adjustments in the workplace, identifies and manages psychosocial hazards and risks."

What is sought is a workplace risk assessment, not a medical opinion. A doctor can advise on me; a doctor cannot assess the Health Service's reporting arrangements.

→ **FOR THE HEALTH SERVICE: a risk assessment of the Switchboard work environment addressing the psychosocial hazards identified. Clause 7.2.1 of the Queensland Public Health Sector Certified Agreement provides the mechanism.**

→ **THEN FOR MY PRACTITIONER: "Are any adjustments recommended in relation to supervision arrangements, and if so, what are they in functional terms?"**

4.3 QUESTION 6 — what are the genuine occupational requirements?

The question asks whether I can "fulfill the full inherent requirements of [my] Role ... without restrictions or modifications to duties."

This applies the wrong test. G03 states its purpose as assisting employees "to meet the genuine occupational requirements of their role by applying principles of reasonable adjustment," and at clause 3 provides that adjustments "should support the employee to undertake the genuine occupational requirements of their role, without risk to their own health and safety or that of others in the workplace." Section 34 of the Anti-Discrimination Act 1991 is to the same effect. Neither asks whether an employee can perform a role without adjustment.

A clinician also cannot determine which duties are genuine occupational requirements. That is the employer's determination. The Health Service has itself already made part of it: its letter states that "established process for client complaints received by Switchboard employees is for immediate escalation and management by the Health Service Client Liaison Officer and/or Manager, Switchboard Services," and the Role Description at Attachment 2 contains no complaint-handling responsibility.

→ **FOR THE HEALTH SERVICE: identify which duties of the role it says are genuine occupational requirements, having regard to the Role Description at Attachment 2.**

→ **THEN FOR MY PRACTITIONER: "Can the employee perform the genuine occupational requirements of the role with the adjustments you have recommended?"**

4.4 QUESTION 9 — can the adjustments be accommodated?

The question asks: "If we are not able to accommodate the restrictions you have recommended, is Mr Shepherd able to safely return to the workplace?"

This presupposes an inability to accommodate before any accommodation has been assessed. G03 clause 2 provides that unjustifiable hardship "is tested against the whole organisation, not a division or unit within the organisation," and that "the onus is on Queensland Health, as the employer, to prove an adjustment is unreasonable, not on the person to prove that it is reasonable."

The question therefore asks my doctor to answer the final step first, on a hypothetical the Health Service has constructed and not yet tested, and on which the onus is not mine.

→ **FOR THE HEALTH SERVICE: assess the recommended adjustments against the whole organisation. If it considers any cannot be accommodated, identify which and its reasons.**

→ **THEN, IF NECESSARY, the question can be put to my practitioner properly.**

PART 5 — QUESTIONS I ASK BE WITHDRAWN, AND A CORRECTION OF FACT

5.1 WITHDRAWAL REQUESTED

Q1(a) "When was Mr Shepherd first diagnosed with MDD?" I do not object to the Health Service being informed of when the diagnosis was made, and I have no wish to appear to withhold it. My objection is to the question as framed. A date standing alone cannot identify a required adjustment or assess the suitability of a proposed one, and so does not meet the limit in G03 clause 3. Nor is a bare date clinically informative without the circumstances in which the diagnosis was made and the course of the condition since. **PROPOSED INSTEAD: "What is the employee's current condition and prognosis, including when the diagnosis was made and the course of the condition since, and over what period are the recommended adjustments expected to be required?"** Framed that way, the Health Service receives the information it seeks together with the clinical context which makes it meaningful, from the practitioner rather than from me, and the question serves the stated purpose. I am content for that question to be put.

Q1(b) "What is the clinical basis for identifying these specific workplace stressors as contributing to the exacerbation of Mr Shepherd's condition." For the purpose of identifying controls, what matters is that the stressors exacerbate my condition and what controls address them — both obtained by Questions 1(d) and 8. The clinical reasoning as to causation is not necessary for that purpose and is a matter in issue in WC/2024/227.

Q1(c) Whether the stressors are based "solely on Mr Shepherd's self-report, your own clinical assessment, or other medical information/reports available to you." This goes to the reliability of the causal history rather than to any adjustment. To the extent it seeks to identify other medical reports concerning me held by third parties, I do not consent to that inquiry.

I record that section 124(1) of the Anti-Discrimination Act 1991 provides that a person "must not ask another person, either orally or in writing, to supply information on which unlawful discrimination might be based," and that under section 124(3) the defence requires the respondent to prove "that the information was reasonably required for a purpose that did not involve discrimination." Impairment is a protected attribute. I raise this not as a complaint but so that the Health Service can satisfy itself that each question it presses meets that standard.

5.2 CORRECTION OF FACT

Question 2 is premised on the statement that "the Health Service is not aware of any concerns being raised for appropriate management."

That statement is incorrect. Concerns have been raised in writing on multiple occasions, including a public interest disclosure (PID 24-ESU-1130); a written patient safety report; correspondence to Human Resources over a period exceeding two years concerning fatigue, rostering and the management of directory information; a formal complaint in February 2026; and the matters in WC/2024/227, in which the Health Service filed an objection dated 5 June 2026 under the signature of the Health Service Chief Executive.

I do not ask the Health Service to agree with any of those concerns. I ask only that my treating practitioner not be asked to answer a question which assumes they were never made. I request that the correction be provided to Dr Ma in writing before he is asked to answer any question resting on that premise.

PART 6 — DELEGATION AND CONFLICT OF INTEREST

I previously raised by email the question of who is making decisions about me and whether any conflict of interest has been identified. I have not received an answer. I ask again, and ask that it be answered in writing.

Both letters of 31 July 2026 are written in the first person of the Injury Management Consultant — "I am seeking further clarification", "I require further information", "I will make a decision regarding your ability to perform your role" — and are signed by Mr Hughes. Please confirm:

- (a) who is the delegate for any decision concerning my capacity or my return to duty;
- (b) the source and instrument of that delegation; and
- (c) whether any conflict of interest has been identified, declared and managed in relation to that decision-maker, and if so, how.

I raise (c) because Question 3 asks my general practitioner to advise whether I am fit to return "under the existing reporting arrangements, including working with and reporting to [my] current line manager." Those reporting arrangements sit within Corporate Services, and the line manager reports to the signatory. The same officer is asked at Part 4 of this document to determine whether those arrangements are safe and whether adjustments to them can be accommodated.

The Manager, Switchboard Services — who on 2 July 2026 required the Employee Capabilities Checklist and advised that my return to work could not be facilitated until it was provided — reports within Corporate Services. The signatory of the letters of 31 July 2026 is the Director of that directorate. The officer now determining my return to duty is therefore the officer to whom the manager who issued that direction reports.

I therefore also ask:

- (d) whether the direction of 2 July 2026, and the decision of 3 July 2026 by which I was excluded from the workplace, were made with the knowledge or approval of the officer now determining my return to duty.

If a conflict is identified, I ask that any decision concerning my capacity or my return to duty be made by an officer outside Corporate Services. Metro South Health is a large organisation and I do not anticipate that this would present

any difficulty.

I make no allegation against any individual. I ask that the position be confirmed, consistent with clause 1.2 of the Code of Conduct for the Queensland Public Service, which provides that a conflict of interest is not wrongdoing in itself, but that failing to disclose and manage one is likely to be wrongdoing. I note that the Managing the risk of psychosocial hazards at work Code of Practice 2022 identifies poor organisational justice — including a lack of procedural fairness — as a psychosocial hazard in its own right, and that G03 clause 1 requires proper consideration to be given to that Code in applying this process.

PART 7 — PROPOSAL FOR RETURN TO WORK

I have been held out of the workplace since 3 July 2026 without wages, with my accrued leave debited, and self-funding the medical costs of this process. G03 clause 1.1 contemplates temporary adjustments which allow an employee to "continue work during their recovery," and clause 1.1.1 confirms that workplace rehabilitation support is available to employees "who have an injury/illness impacting their current ability to perform their role, with no claim or entitlement to compensation (health management case)." Clause 1 provides that "Managers are responsible for ensuring work environments and work, do not unreasonably prevent employment for people."

I propose the following. Each item is within the Health Service's existing processes and, so far as I can see, requires no material expenditure and no restructuring.

- (a) RETURN TO DUTY. That I return to my rostered shifts now, on the restrictions certified in the Employee Capabilities Checklist of 3 July 2026, while the balance of this inquiry is completed. If the exclusion is maintained, that the instrument relied on be identified, that the period be treated as paid special leave, and that the leave debited since 3 July 2026 be reversed.
- (b) COMPLAINT HANDLING. That the Health Service confirm in writing that complaints received by Switchboard staff are escalated in accordance with the process it has itself described. This requires no change to anything.
- (c) ROSTER PREDICTABILITY. That my roster be published in accordance with the certified agreement and not altered without my agreement other than in an emergency.
- (d) ORDINARY PROCEDURAL FAIRNESS. That any concern about my performance or conduct be raised with me in writing, with particulars, in accordance with the applicable directive, and that I be afforded a support person.
- (e) A SEPARATE CONTACT POINT. That leave, injury management and human resources matters concerning me be handled by an officer outside Switchboard Services rather than by my line manager, who wrote to me on 2 July 2026 concerning leave types and my return to duty. This does not alter day-to-day supervision and involves no change to reporting lines.
- (f) WORKPLACE RISK ASSESSMENT. As set out at Part 4.2.
- (g) REVIEW. That the arrangements be reviewed at six weeks, or earlier if either party considers it necessary.
- (h) GRADUATED RETURN. If the Health Service prefers, I am willing to return on a graduated basis by agreement. I am not seeking to avoid any part of my role.

If any part of this proposal cannot be accommodated, I ask that the Health Service identify which part and its reasons, having regard to G03 clause 2.

PART 8 — TIME

The request allows seven calendar days from receipt, expiring 7 August 2026, and expressly contemplates a reasonable extension. I request one.

The letter was received at 11:43am on Friday 31 July, approximately two hours before a consultation with Dr Ma of which the Health Service had been notified on 30 July. Nine questions cannot be put to a practitioner on two hours' notice, and because the scope of the questions is in issue I cancelled that consultation rather than incur a cost that both the Health Service and I would have wasted. I will rebook it once the question set is settled.

I propose that the response be provided in two parts:

- (a) the questions within Dr Ma's scope — 21 days from the date the Health Service confirms the final question set; and
- (b) any question requiring specialist input — on a timeframe set once the Health Service confirms which questions it presses and that it will meet the cost.

I do not seek delay; the exclusion is costing me my income. I ask the Health Service to note that several of its questions can only be answered by a specialist, and that pressing them necessarily extends the inquiry. It cannot be reasonable to maintain an unpaid exclusion for a period created by the Health Service's own requirements.

PART 9 — DISCLOSURE AND COSTS

DISCLOSURE. I adopt option 1 and will obtain the information from my practitioners and provide it. I do not consent to option 2 and do not provide an authority for the Health Service to obtain information directly from my treating practitioners. G03 clause 3 provides that "Queensland Health acknowledges the right of the person with disability, condition, cultural considerations or neurodivergence to choose how disclosure of information occurs," and that "it may only be necessary to share information when the ability to meet genuine occupational requirements and or individual safety is at risk and reasonable adjustment is required." My election is the exercise of a right the policy expressly recognises.

COSTS. I note and accept the confirmation that "MSH will meet your reasonable costs of preparing this report."

- (a) The consultation of 3 July 2026 which produced the Employee Capabilities Checklist was invoiced at \$150.00 and paid by me on 10 July 2026 (receipt N471065713633). I ask that it be reimbursed.
- (b) The rebooked consultation with Dr Ma, and any further consultation required to answer the Health Service's questions, will be invoiced to the Injury Management Consultant at the address nominated.
- (c) If specialist input is required, please confirm the Health Service will meet that cost on the same basis.

RECORDS. The covering email is copied to notes@solv.com.au and refers to a transition to "Solv, a new Injury Management Case System." Please confirm what personal and health information concerning me is held in or transmitted to that system, the basis on which it is held, and the arrangements governing its use and disclosure.

PART 10 — RESPONSE REQUESTED

I ask for a written response confirming:

1. whether I am returned to my roster, and if not, the instrument relied on to maintain my exclusion and whether the period will be treated as paid special leave with my leave re-credited;
2. the delegate, the source of the delegation, how any conflict of interest has been identified and managed, and — if one is identified — that the decision be reassigned to an officer outside Corporate Services;
3. whether any performance or conduct process concerning me is on foot or contemplated;
4. which duties of my role the Health Service says are genuine occupational requirements;
5. whether the Health Service will conduct the workplace risk assessment;
6. which questions it presses, which it withdraws, and whether it will meet the cost of any specialist opinion; and
7. its response to the proposal at Part 7.

I confirm again that I am engaging with this process and want to return to work.

Cory Lea Shepherd AO3, Switchboard Services, Logan Hospital 0417 400 227 · coryshepherd1@hotmail.com